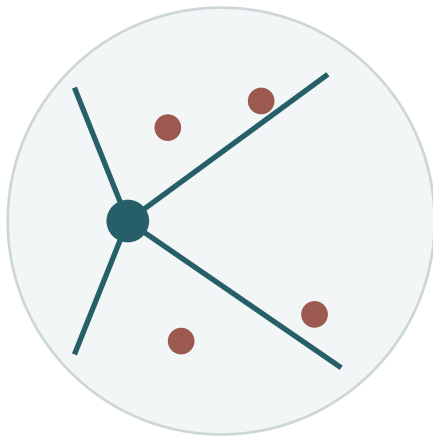


Retinal vein occlusion ischaemia and surveillance

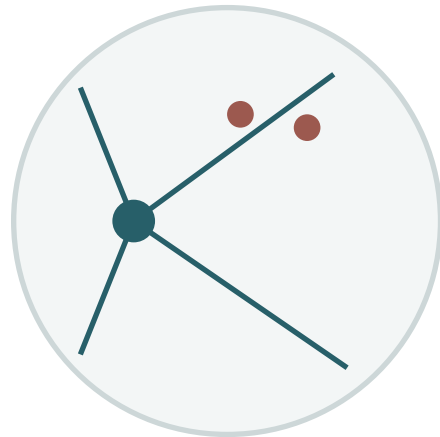
Part 2 | Distribution and mechanism

The affected territory helps classify the occlusion.

CRVO: diffuse



BRVO: sectoral



Distribution guides interpretation; perfusion still requires functional and imaging assessment.

Original fundus map: diffuse versus sectoral haemorrhagic distribution.

A retinal vein occlusion is not one homogeneous prognostic group. Document visual function, haemorrhage distribution, pressure and vascular risk. Angiographic findings must be interpreted with haemorrhage masking and the interval since presentation in mind.

Oedema and ischaemia are different outcomes

A dry OCT does not prove restored retinal perfusion.

Finding	Implication
Oedema	OCT fluid and thickness; potentially treatable
Macular ischaemia	May limit function despite reduced fluid
Peripheral nonperfusion	Drives retinal or anterior-segment neovascular risk
Treatment response	Reassess function and perfusion separately

Do not interpret central thickness as a complete outcome measure. Treatment may reduce leakage without removing the ischaemic drive. Ask whether further therapy can improve the remaining mechanism or mainly changes a scan that no longer explains function.

Surveillance during and after anti-VEGF

Visible neovascularisation may be suppressed rather than prevented.

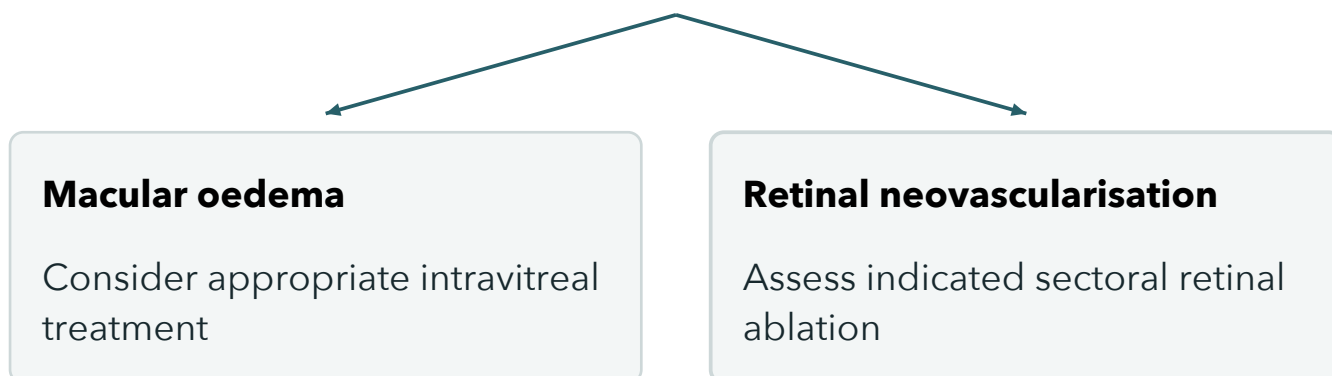
- Reassess retinal ischaemia and current treatment
- ↓
- Examine iris and angle when risk remains
- ↓
- Plan review when injections are extended or stopped
- ↓
- Act promptly on new neovascular signs

The monitoring plan is part of treatment. Explain why a patient may still need visits when central vision has improved and injection frequency falls. A change in attendance can alter the risk of missing a treatable neovascular complication.

BRVO: choose the target

Treat the complication rather than the photograph alone.

What is limiting vision or threatening the eye?



Retinal collaterals and new vessels are not interchangeable findings. A management answer should specify the pathological target, its distribution and the intended outcome. Avoid a global PRP rule for every vein occlusion without examining the actual phenotype.

Systemic review and atypical patterns

Use a targeted systemic assessment rather than indiscriminate testing.

Finding	Implication
Common context	Blood pressure, metabolic risk and glaucoma assessment
Young or bilateral disease	Review unusual systemic and ocular causes
Inflammatory signs	Consider vasculitis and relevant directed investigation
Abnormal history	Coordinate further testing with the appropriate specialist

The useful examination response links a specific atypical finding to a specific investigation. Age alone does not dictate a large undirected panel. State how a test result would alter diagnosis, systemic referral or management.